



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : مرزوقه ناصر محمد عبد المنعم

العنوان : اخميم

الحي / المدينة : اخميم

المحافظة : سوهاج

تاريخ تسجيل الإستمارة : 09-10-2026

رقم البطاقة : 28203232601527 رقم التليفون : 01155222405

العمر (تاريخ الميلاد) : 23-03-1982 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: مرزوقه ناصر محمد عبد المنعم National ID: 28203232601527	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM 10/1/2025 G

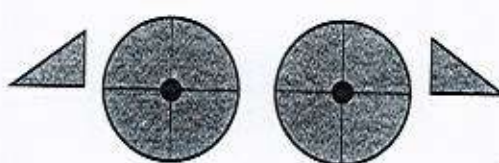
<u>Name</u>	Mar 2015/10	<u>MRN</u>	CCOV-000
<u>Age</u>	44 yrs.	<u>ID</u>	SNP-PCVCT-1000
<u>Menopausal status</u>	pre-menopausal	<u>Phone</u>	-11000000-0
<u>NRS SCORE</u>		<u>Marital status</u>	
<u>BMI</u>		<u>Family history</u>	
<u>CO morbidity</u>		<u>REASON OF MDT</u>	Re mdt

Clinical Data

<u>BX:</u>			
<u>ER:</u> 8/8	<u>PR:</u> 8/8	<u>HER 2:</u> 1	<u>Ki67:</u> 72

Sonomammography:

Mets Workup:



TNM

Recommendations:

It breaks into $\rightarrow$ 11 BCS (1224) $\rightarrow$ ATZ N. M.
 $\rightarrow$ AC x 4 $\rightarrow$ Taxider x 4 $\rightarrow$ All 12 blades $\rightarrow$ 12 blades

Pt is having regressive senses

MDT OUTCOME & PLAN

Consider Zolser 9-10 wks

⑤ shift to noindex

STAD Policy - TAME, Social Skills

3. 1/2 months

SIGNATURE

the Greek, etc

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: مرزوقه ناصر	
MR. : 2973	
Detailed Medical Report	
السيد	
تحتية طبية وبعد,,	
	إيماء للطلب المقدم من بشأن تقرير طبي للمريض
	بقسم: رقم طبي:
ننشر بأن نوضح فيما يلي تقريراً عن حالته	
	تاريخ وساعة الدخول : تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient is currently receiving Zoladex and Tamoxifen and has no acute distress on examination. The patient was started on letrozole and Zoladex due to vaginal bleeding. A CT of the chest, abdomen, and pelvis demonstrated a free, tiny, insignificant nodule; breast imaging was reported as BIRADS 1 while the patient continues letrozole therapy. The patient reports post-coital bleeding; mammosonography performed in June 2026 was BIRADS I, and CT CAP shows multiple pulmonary nodules, the largest measuring 4.5 mm, which were also present on the 2024 CT. The patient remains on Zoladex and Tamoxifen, not on an aromatase inhibitor (menstruation was noted while on AI). The plan includes gynecological consultation, transvaginal ultrasound, and estradiol level assessment; continue Zoladex/Tamoxifen and add zoledronic acid after dental assessment. The patient is free for a new CT CAP and zoledronic acid may be considered every six months after dental examination.	
تحريراً في ,	
	التاريخ : 18-09-2026
	الوقت : محمد رضا أبو اليزيد كيلاني
الطبيب المعالج :	
يعتمد ,	



Patient Name: Marzoukah Nasser Mohamed Abdel Na'eem	Date : 20/08/2024
Exam : C.T CHEST WITH CONTRAST	Age : 42 Year(s)
Medical ID: 2257/2024	Pat. ID: 54060

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

1. Chest:

- Left breast UOQ heterogenous fibro-glandular tissue.
- Multiple bilateral scattered tiny pulmonary nodules predominately in right lung, the largest one about 5 mm seen in lateral segment of right middle lung lobe
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- RVF uterus with left adnexal small cystic lesion reaching 1.5 cm for regional US assessment.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.



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3. The scanned skeleton:

- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Reported case of breast cancer showing:

- *Bilateral scattered pulmonary nodules of indeterminate nature for close follow up.*
- *No definite hepatic metastatic deposits.*

Signature Dr Ahlam Shamsan

20/08/2024 12:38 PM

Dr. Mohamed Mohsen, M.D.

Patient Name : الأستاذ / مرزوقه ناصر محمد عبد النعيم
Patient No. : 28203232601527
Birth No. : 3986/24
Sex : Female
Age : 42 Y
Referring Doc :

Request No. : 13009464
Request Date : 17/10/2024
Verification Date : 29/10/2024
Financial Category : أورام دار السلام - مبادره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Up-Front.
Nature Specimen: Left simple mastectomy and SLNs.

Gross Examination:

- * Procedure: Left simple mastectomy specimen measured 22x19x5 cm with covering skin ellipse measured 19x13.5 cm.
- * Tumor extent: A greyish white mass.
- * Size: 4x4x2cm and rubbery grayish white area measured 8x6.5x2 cm.
- * Skin involvement: Unremarkable grossly.
- * Distance from margins: 1.5 cm away from deep margin and 1.5cm away from the skin
- * Lymph nodes number and size: Fibrofatty tissue peice measured 4x4 cm entangling 5 LNs up to 2.5 cm.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
- * Angioinvasion: Positive.
- * Ductal carcinoma in situ: Focal (20%)(comedo, solid and cribriform)
- * Skin involvement: Free nipple and areola.
- * Lymph node involvement: Positive in (1/5) with no extracapsular extension.
- * Resection Margins: Free deep margin.
- * Microcalcifications: Positive.
- * Additional pathologic findings: Rest of breast show adenosis, fibrosis, apocrine metaplasia and fibrocystic disease.

Frozen section results:

Positive SLNs (1/5).

Verified by :
Dr. Sagda El-Kady

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Page 1 of 2

Printing By : .marwa mohamed youss

CAP
ACCREDITED

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name الأستاذ / مرزوقه ناصر محمد عبد النعيم
Patient No. 28203232601527
Birth No. 2141/24
Sex Female
Age 42 Y
Referring Doc.

Request No. 13009109
Request Date 29/06/2024
Verification Date 14/07/2024
Financial Category اورام دار السلام - ميانره
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or incomplete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Prof. Dr. Aml Hareedy.

Dr. Heba Hassan.

Verified by :

Patient Name	: الأستاذ / مرزوقه ناصر محمد عبد النعيم	Request No.	: 13009464
Patient No.	: 28203232601527	Request Date	: 17/10/2024
Path No.	: 3986/24	Verification Date	: 29/10/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 42 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Diagnosis:

Left breast, simple mastectomy and SLNs Biopsy:
Invasive duct carcinoma, Grade II with DCIS, positive lymphovascular emboli and
microcalcification.
Free deep margin.
Free nipple and areola.
Positive lymph nodes (1/5) with no extracapsular extension.
Stage: pT2, pN1a.

Signature

Dr. Salma Tarek.

Dr. Sagda Elkady.

Verified by :

Dr. Sagda El-Kady

Printing Date : 30/10/2024 10:24:43

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Marzoukah Nasser Mohamed Abdel Na'eem	Date : 22/05/2024
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical ID: 28203232601527	Pat. ID: 54060

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Left breast palpable lump.

Report:

- ACR b: scattered fibroglandular parenchyma
- Left Breast:
 - The UOQ focal asymmetry corresponding by US heterogenous altered parenchyma extending from 12 -2 o'clock measuring about 1.5x 4 cm which is seen surrounded by irregular heterogenous small nodules seen extending from 1-3 o'clock the largest measures about 0.8 cm seen at 3 o'clock. The pathological process is seen measuring collectively 73x41x59mm (in MLO view).
 - Multiple prominent ducts are seen related to the aforementioned lesion, raising the possibility of intraductal extension.
 - Few (about 2) enlarged indeterminate level axillary lymph node showing focal cortical thickness measures 3mm.
 - No supra or infraclavicular lymph node.
- Right Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex.
 - Mildly dilated retroareolar ducts with internal clear contents.
 - Few subcentimetric UOQ anechoic cyst.,
 - Nonspecific axillary lymph node.

Patient Name	: الأستاذ / مرزوقه ناصر محمد عبد النعيم	Request No.	: 13009109
Patient No.	: 28203232601527	Request Date	: 29/06/2024
Path No.	: 2141/24	Verification Date	: 14/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 42 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 29/6/2024, 10:30 am.

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

IPT (Positive control showed appropriate reactivity):

ER: Strong nuclear reaction in 100% of tumor cells (score 8/8).

PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).

Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).

Ki67: 7% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

PT:

ER: Positive (score 8/8).

PR: Positive (score 8/8).

Her2: (Score 1+).

Ki67: Positive in 7% of tumor cells.

Scoring system: "ER, PR"

SCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, 1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between

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Page 1 of 2

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Patient Name الأستاذ / مرزوقه ناصر محمد عبد النعيم
Patient No. 28203232601527
Birth No. 2141/24
Sex Female
Age 42 Y
Referring Doc.

Request No. 13009109
Request Date 29/06/2024
Verification Date 14/07/2024
Financial Category اورام دار السلام - مياذره
Department Out Patient

Registered by .marwa mohamed yousse

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Signature

Prof. Dr. Aml Hareedy.

Dr. Heba Hassan.

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Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name: Marzoukah Nasser Mohamed Abdel Na'eem	Date : 22/05/2024
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical ID: 28203232601527	Pat. ID: 54060

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Left breast palpable lump.

Report:

- ACR b: scattered fibroglandular parenchyma
- Left Breast:
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 - Few subcentimetric UOQ anechoic cyst.,
 - Nonspecific axillary lymph node.

Patient No. : 2257-2024
Patient Name : الأستاذ / مرزوقه ناصر محمد عبد النعيم
Sex : Female Age: 43 Y
Financial Cate : جوستاف روسي - نفقة الدولة
Department : Out Patient
Referring Doc :

Request No : 13009969
Request Date : 04-08-2025 12:37
Verification Date : 04-08-2025 14:50
Sample Type : Serum
Test Status : Routine
Analyzers : Mini_Vidas_Hermel

Hormones&Tumor Markers Report

Test	Result	Unit	Ref. Range
Estradiol	19.24	pg/mL	Follicular : 10-160 Luteal : 27-246 Midcycle : 34-400 Post menoposal: ND-30

Signature

Dr./Manal Mokhtar

Patient No. : 2257-2024
Patient Name : الأستاذ / مرزوقه ناصر محمد عبد النعيم
Sex : Female Age: 43 Y
Financial Cate : جوستاف روسي - نفقة الدولة
Department : Out Patient
Referring Doc :

Request No : 13009969
Request Date : 04-08-2025 12:37
Verification Date : 04-08-2025 14:08
Sample Type : Serum
Test Status : Routine
Analyzers :

Clinical Chemistry Report

Test	Result	Unit	Ref. Range
Cholesterol	186.0	mg/dL	Up to 200.0)
Triglycerides	<u>170.0</u>	mg/dl	35.0 - 160.0)

Signature

Dr./Manal Mokhtar

Printing Date : 07/08/2025 10:27:35

Page 1 of 1

Printing By : hermal

25311121 / فاكس

25311128 - 25311125 / ت

987 كورنيش النيل الملك الصالح

Patient Name: Marzoukah Nasser Mohamed Abdel Na'eem	Date : 22/07/2025
Exam : ultrasound TRANS VAGINAL	Age : 43 Year(s)
Medical ID: 22257/2024 260	Pat. ID: 54060

Trans-vaginal ultrasound scanning revealed: -

○ **Uterus: -**

- AVF, average sized uterus for age..
- Average endometrial line = 5 mm .
- IUCD is seen in place.
- The cervix shows multiple nabothian follicles.

- No gross adnexal soft tissue masses or cysts.
- No free fluid in the Douglas pouch.

Conclusion:

Normal examination.

Best regards

Dr/Rasha Elbrahim



To whom it may concern

(Mrs./ Mr .. Marzouk Mohamed .., 44 years old

(female/ male patient presented with diagnosis

left Breast Ca

(She/ He was referred to us for (radical/ adjuvant / Neoadjuvant /
palliative) Radiotherapy

(She/ He has received (3D / IMRT) radiotherapy to.....

left Chest wall + SC

At Dose : 40 Gy/ 15 fx/ 3 w .

Additional (Boost) ; yes/ no, at dose Gy/ fx/ w

(sequential / concomitant)

Started: 11 / 8 / 2025

Ended: 30 / 8 / 2025 With good tolerance .

Head of department
Dr. Hany William